

Anshumaan Saraf

Forward Deployed Engineer — Heidi Entry Program

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What I Built

Heidi Voicemail Intelligence (HVI) — An end-to-end intelligent voicemail triage dashboard built on top of Heidi Health. It turns raw after-hours GP voicemails into prioritised, structured, actionable work items for clinic admin staff.

Live demo: master.d14xijxbarple1.amplifyapp.com

The Problem I Saw

Multi-location GP clinics using Heidi Calls receive 20-40 unstructured voicemails overnight. Every morning, admin staff arrive to a flat list with no urgency signals, no structured data, and no way to prioritise without listening to each recording in full. Before the clinical day begins, they are already behind. This is not an audio problem. It is a workflow and information problem.

What I Shipped

HVI processes each voicemail through an AI pipeline (Deepgram transcription to Claude classification) and surfaces every recording as a structured card with:

- **Urgency classification** — URGENT / HIGH / NORMAL / LOW, auto-ranked with confidence scores
- **Intent detection** — 14 standardised codes (rx-refill, appt-book, symptom-acute, etc.)
- **Plain-English summaries** — 1-2 sentence digests so admin never has to listen blind
- **Key detail extraction** — dates, medications, symptoms pulled automatically
- **Suggested actions** — what the admin should do next, surfaced inline
- **Full transcript + audio playback** — original recording always one click away
- **Multi-clinic filtering** — Clinic 1 / Clinic 2 / Clinic 3 with cross-location stats
- **Status workflow** — New, In Progress, Done with full audit trail

Architecture & Tech Stack

Full-stack application: **Next.js 15** (App Router, SSR), **TypeScript 5**, **Tailwind CSS v4** with Heidi design tokens, **Radix UI** primitives, **Neon serverless Postgres** for persistence, deployed on **AWS Amplify**. The AI pipeline uses **Deepgram Nova-2** (Australian English, speaker diarization) for transcription and **Claude** for structured JSON classification — urgency, intent, summary, key details, and suggested action in a single pass.

I designed the data model (voicemails + status_events tables), built 12 realistic seed voicemails with real audio recordings, implemented the three-panel layout (sidebar, list, detail) matching Heidi's existing design language, and deployed it live. Cost model: approximately \$0.26/night for 30 voicemails on the Deepgram side.

Why This Project Exists

I did not build HVI because someone asked me to. I built it because I looked at Heidi's product surface — Scribe, Evidence, Calls — and saw an obvious gap: after-hours voicemails are a high-volume, zero-structure workflow that Heidi Calls enables but does not yet triage. Admin staff are the bottleneck, and they have no tools. I wanted to show, not explain, what a solution could look like.

How I Think About Building

I am not a polished engineer with years of production experience. I am someone who sees a problem, picks a direction, makes tradeoffs, and ships something real. Here is what that looked like on this project:

Product thinking first. I started with the user — clinic admin staff arriving at 7:30 AM to 30+ voicemails. I mapped their workflow before writing a line of code. Every feature traces back to a pain point: urgency badges exist because they should never have to listen to a routine appointment request before a chest pain callback.

Design decisions, not decoration. I used Heidi's actual brand tokens (#4c2934, #f7f1ee) and replicated their sidebar navigation pattern so HVI feels native to the platform. This was a deliberate choice — I wanted to demonstrate what a real feature integration would look like, not a disconnected side project.

Real infrastructure. I did not mock a database with JSON files. I set up Neon Postgres, wrote the schema, built seed scripts, and deployed to AWS Amplify with a CI pipeline. The app is live and functional, not a Figma prototype.

Costed the AI pipeline. I scoped Deepgram Nova-2 at ~\$0.0043/min and calculated the nightly cost for a typical clinic. This is how I think — not just 'can we build it' but 'does the unit economics work for a 3-clinic practice?'

What I Am Learning — And Where I Am Going

I believe I am startup material. I do not say that because I have a track record — I say it because of how I am wired. I have spent the past months going deep into AI healthcare, not because someone assigned it, but because I genuinely believe this is where the highest-leverage work is happening.

Along the way, I have been teaching myself everything the job demands — from branding (I matched Heidi's design system by hand) to marketing (understanding PLG funnels and how Heidi's freemium model drives adoption), to the product itself (mapping Scribe, Evidence, Calls, and where the gaps are), to the technology layer (LLM integration, RAG patterns, prompt engineering, ASR pipelines, cloud deployment). I still do not know everything. But I am paving my way, and I learn fast.

What I want from the Entry Program is a place where that intensity is an asset, not an anomaly. A place where I can be thrown at real problems alongside people who build for a living. I do not need hand-holding. I need reps.

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Heidi Health

Entry Program — Forward Deployed Engineer

Dear Heidi Team,

You asked applicants to pick a direction, make tradeoffs, and ship something tangible. So I did. I built Heidi Voicemail Intelligence — a full-stack AI triage dashboard that turns unstructured after-hours GP voicemails into prioritised, actionable work items for clinic admin staff. It is live, deployed, and functional at master.d14xjxbarple1.amplifyapp.com.

I chose this problem because it sits at the intersection of what Heidi already does well (ambient AI for clinicians) and a gap I identified in the workflow: after-hours voicemails are high-volume, zero-structure, and entirely manual. Admin staff have no urgency signals, no structured data, and no way to prioritise without listening to every recording. HVI solves that with Deepgram transcription, Claude classification, and a three-panel dashboard that sorts voicemails by urgency before anyone picks up a headset.

I am not applying because I have years of experience. I am applying because I have the instinct to look at a product, find the unsealed edge, and build something that closes it — without being asked. Over the past months, I have gone from zero to shipping a production-grade Next.js application with a real database, real audio, real AI classification, and real deployment infrastructure. I taught myself the stack, matched Heidi's design language, costed the AI pipeline, and deployed it live.

As a Forward Deployed Engineer, I want to be the person who makes Heidi actually work in the real world — embedded with strategic customers, solving technical onboarding problems, designing integration workflows, and making sure the product delivers value in clinical settings. I understand that the specific skills can be taught quickly. I am here to prove the underlying hardware is already there.

I do not have a perfect resume. What I have is a live application that demonstrates how I think, how I build, and how far I can push when I care about the problem. I would welcome the chance to bring that energy to Heidi.

Warm regards,

Anshumaan Saraf